

APRIL 17TH STUDY GUIDE

CHES Study Guide — Area III: Implementation and Management

Key Concepts and Knowledge Checks

FIDELITY AND TRAINING PROTOCOLS

These two concepts are tightly connected. Fidelity is the goal, and a training protocol is the tool you use to achieve it. They show up together on the CHES exam because implementation quality depends on both.

Fidelity

Fidelity means delivering a program **exactly as it was designed**. Same content, same sequence, same methods — no freelancing. When a health education curriculum is implemented by a single trained facilitator instead of letting each classroom teacher do their own version, that's a fidelity decision. The whole point is consistency: every participant gets the same intervention, which means your evaluation results are actually meaningful. If everyone delivers the program differently, you can't know what worked.

Why this matters on the exam:

Fidelity questions often set up a scenario where someone wants to change how a program is delivered — teachers want to teach it themselves, staff want to skip sections, a site wants to modify the curriculum. The correct answer is almost always the one that explains why the program needs to be delivered as designed. Don't confuse fidelity with validity (whether something measures what it claims to measure) or reliability (whether results are consistent across measurements).

Training Protocol

A training protocol is the **standardized guide** that tells program staff exactly how to deliver an intervention. It includes the procedures, scripts, activities, and materials that ensure every facilitator delivers the program the same way. If a question asks what should be used to standardize intervention delivery by program staff, the answer is a training protocol — not a motivation model, not a learning motivation framework, and not an evaluation plan.

How they connect:

Fidelity is the **principle** (deliver it as designed). A training protocol is the **tool** (the document that makes fidelity possible). You train staff using a protocol so they maintain fidelity when they deliver the program.

Knowledge Checks:

1. A community health program trains four different facilitators to deliver a diabetes prevention curriculum across four sites. Each facilitator follows the same lesson plans, uses the same materials, and delivers sessions in the same order. This consistency is an example of what concept?
2. A school district wants to let each health teacher modify an evidence-based drug prevention curriculum to fit their teaching style. The program developer pushes back. What concept is the developer protecting?
3. A new staff member is about to deliver a tobacco cessation program for the first time. The program manager hands them a document with step-by-step procedures, scripts, and a list of required materials. What is this document called?
4. Which of the following would best standardize how an intervention is delivered across multiple program staff: a learning motivation framework, a training protocol, an evaluation plan, or a motivation model?

PROGRAM SUSTAINABILITY

Sustainability is about whether a program can **keep going** after the initial funding, energy, or support runs out. It's a management issue, not a planning issue — it shows up when you're already running a program and realize you might not be able to continue.

What sustainability looks like:

A program is three-quarters through its annual budget and realizes the money won't last the rest of the year. That's a sustainability problem. It's not about whether the program was well-planned (that's planning), whether it's being evaluated (that's evaluation), or whether it will outlive its founders (that's longevity in a general sense). Sustainability specifically means: **can you keep the program operating with the resources you have or can secure?**

Exam tip:

The distractors for sustainability questions are usually "longevity," "evaluation," and "planning." Longevity is close but it's a general term, not a program management concept. If the scenario describes a funding shortfall or resource gap mid-program, the answer is sustainability.

Knowledge Checks:

1. A nonprofit is halfway through a grant-funded nutrition education program when the grant sponsor announces they will not renew funding next year. The program director begins seeking alternative funding sources. This is an issue of program what?
2. A health education specialist has been running a successful walking program for two years. The community center that provides free space announces it's closing. The specialist starts looking for a new venue. What program concern is this?
3. What is the difference between program sustainability and program longevity?

ADULT LEARNING PRINCIPLES

Adult learning principles come from andragogy — the theory of how adults learn, developed by Malcolm Knowles. Adults learn differently than children, and health education specialists need to design trainings accordingly. The CHES exam tests whether you can identify what IS and what is NOT an adult learning principle.

Core adult learning principles:

Adults are self-directed. They want a voice in what they learn and how. Involve them in setting the curriculum, choosing training methods, and identifying goals.

Adults bring experience. They have prior knowledge and life experience that should be acknowledged and used — not ignored. Ask about their background and avoid redundant content.

Adults need relevance. They want to know why something matters to their life or work right now. Conduct assessments to understand their goals and connect content to those goals.

Adults are problem-centered. They learn best when content is organized around real problems they need to solve, not abstract theory.

What is NOT an adult learning principle:

Using a **prescribed, standardized set of methods and content** to avoid confusion is NOT an adult learning principle — it's the opposite. Standardizing instruction treats adults like passive recipients. Adult learning is about flexibility, engagement, and building on what learners already know. On the exam, the answer that describes a rigid, one-size-fits-all approach is your "NOT an adult learning principle" answer.

Knowledge Checks:

1. A trainer begins a workshop by asking participants what they already know about the topic and what they hope to learn. Which adult learning principle is this?
2. Which of the following is NOT an adult learning principle: (a) involving adults in choosing training methods, (b) using a standardized script to ensure all trainers say the same thing, (c) connecting content to learners' real-world goals, (d) drawing on participants' prior experience?
3. A health education specialist is designing a training for community health workers. To apply adult learning principles, should the specialist let participants help shape the agenda, or should the specialist deliver a fixed, pre-set curriculum with no modifications?

PILOT TESTING AND PHASING IN

Both of these concepts are about how you roll out a program — but they serve different purposes. Pilot testing happens before full implementation; phasing in is a strategy for how you do the full implementation.

Pilot Testing

A pilot test is a **small-scale trial run** of a program or intervention before you launch it fully. The purpose is to find out whether the program will work as designed — does the curriculum make sense? Do the materials work? Are the logistics feasible? You test it with a small group, identify problems, make adjustments, and then roll out the real thing. Think of it as a dress rehearsal.

Phasing In

Phasing in means **implementing a program in stages** rather than launching everything at once. If an intervention targets multiple levels of the social ecological model (individual, organizational, community), phasing in means you start with one level and add the others over time. This is practical when some components take longer to develop or when resources don't allow a full launch all at once. It's not about testing whether something works — it's about managing the rollout.

How to tell them apart on the exam:

Is the question about **testing whether something works** before going live? → Pilot test. Is the question about **rolling out a multi-component intervention gradually** rather than all at once? → Phasing in. A pilot test answers "Will this work?" Phasing in answers "How do we get all of this launched when we can't do it all at once?"

Knowledge Checks:

1. A health education specialist has designed a new after-school physical activity program. Before launching it across all 12 schools in the district, the specialist runs it at two schools for six weeks. What is this?
2. A community intervention targets individual behavior change, organizational policy, and local government advocacy. The team decides to start with individual-level programming this year and add organizational and policy components next year. What is this approach called?
3. A specialist wants to know if a new survey instrument will be understood by participants before using it in a large evaluation. They test it with 20 people first. Is this a pilot test or phasing in?

PROCESS EVALUATION

Process evaluation asks: **"Are we doing what we said we'd do?"** It measures the implementation of a program — not whether the program changed health outcomes, but whether the program was delivered as planned. This is an Area III concept because it's directly tied to implementation and management.

What process evaluation measures:

Fidelity — Was the program delivered as designed?

Completeness — Were all planned activities actually carried out?

Exposure — Did the intended audience actually receive or participate in the program?

These three — fidelity, completeness, and exposure — are the classic process evaluation indicators. If participants are asked whether they find the instructional materials easy to use or whether the sessions are running as planned, that's process evaluation.

What process evaluation does NOT measure:

It does not measure health outcomes (that's outcome or summative evaluation), it does not measure immediate program effects (that's impact evaluation), and it does not measure health status or quality of life (that's also outcome). Process evaluation is about the **machinery of the program**, not the results.

Knowledge Checks:

1. During a health education program, participants are asked whether the instructional materials are easy to use and whether they find the sessions helpful. What type of evaluation is this?
2. A program manager reviews attendance records, checks whether all 10 planned sessions were delivered, and confirms that facilitators followed the curriculum guide. What type of evaluation?
3. What three things does process evaluation typically measure?
4. A funder asks whether the program reduced diabetes rates in the community. Is this a process evaluation question? Why or why not?

TARGETING VS. TAILORING

Both targeting and tailoring are communication strategies used to reach priority populations more effectively. They sound similar but they work at different levels, and the CHES exam expects you to know the difference.

Targeting

Designing a message for a **specific group** based on shared characteristics. Everyone in the group gets the **same message**. The message is crafted to resonate with that group's demographics, culture, age, or health concerns — but it's one message for the whole group. Example: sending breast cancer screening materials to all women ages 40 and older. The message is designed for that group, but every woman in the group gets the same thing.

Tailoring

Customizing a message for a **specific individual** based on their personal characteristics, behaviors, or needs. Each person gets a **different message** (or different version of a message) based on their unique profile. Example: a fitness app that sends personalized daily step goals and encouragement based on each user's current activity level and health data. The content changes from person to person.

How to tell them apart on the exam:

Is the message the same for everyone in a defined group? → **Targeting**. Is the message customized for each individual? → **Tailoring**. Targeting = one message, one group. Tailoring = unique message, one person.

Knowledge Checks:

1. A health department creates a brochure about heart disease prevention written in Spanish and distributed to all Latino community members over age 50. Is this targeting or tailoring?
2. A computer application sends personalized text messages to individual patients based on their fitness level, daily steps, and health goals. Is this targeting or tailoring?
3. A health education specialist sends cancer prevention materials to all women 40 and older about breast cancer screening. Is this targeting or tailoring?

LOGIC MODELS AND INTERVENTION MAPPING

These two show up in Area III because they're tools used during implementation to keep programs on track. A logic model gives you the big picture of how your program is supposed to work. Intervention mapping gives you the detailed step-by-step plan for building it.

Logic Model

A logic model is a **visual diagram** that maps out the relationship between what you put into a program and what you expect to get out of it. The standard flow is: **Inputs** → **Activities** → **Outputs** → **Outcomes**. Inputs are your resources (staff, money, space). Activities are what you do (workshops, screenings, trainings). Outputs are what you produce (number of sessions held, materials distributed). Outcomes are what changes (knowledge gains, behavior changes, health improvements).

What logic models are used for:

They track whether your **activities are producing outputs that actually lead to outcomes**. If a question asks what tool shows the big picture of an intervention's resources, activities, and results — or what tool tracks whether activities are producing outputs leading to outcomes — the answer is a logic model.

Intervention Mapping

Intervention mapping is a **detailed, systematic protocol** for developing health education programs that are grounded in theory and evidence. It walks you through every decision: identifying what needs to change, selecting theory-based methods, designing program materials, and planning implementation. If a question asks about developing a work plan that's specifically aligned to a logic model, intervention mapping is the approach that connects the two.

How to tell them apart on the exam:

Need a **visual summary** of how the program works (resources → activities → results)? → **Logic model**. Need a **step-by-step protocol** for building a theory-based program aligned to that model? → **Intervention mapping**. The logic model is the map; intervention mapping is the directions for building what the map shows.

Knowledge Checks:

1. A program manager wants to show a funder how the program's resources connect to its activities and expected outcomes in one visual. What tool should they create?
2. A health education specialist needs to verify that the workshops being delivered are actually leading to the knowledge gains the program promised. Which tool helps track this connection?
3. A team is building a new evidence-based intervention from scratch and needs a systematic, step-by-step process to ensure every component is grounded in theory. What approach should they use?
4. What is the standard flow of a logic model?

PROCUREMENT AND AGREEMENTS

When programs need outside services — consultants, contractors, partner organizations — the type of agreement you use depends on the nature of the relationship. The CHES exam tests whether you know which agreement fits which situation.

Memorandum of Understanding (MOU)

An MOU is used when you have a **long-term partnership** where both parties agree to work together, often with mutual benefits. It's less formal than a contract and typically doesn't involve a competitive bidding process. If a long-term program partner can provide services at a discounted price, an MOU is the appropriate agreement — it documents the partnership terms without the rigidity of a formal contract.

Formal Contract

A legally binding agreement with **specific deliverables, timelines, and payment terms**. Used when you need to hold a vendor or contractor accountable for defined work. More rigid and legally enforceable than an MOU.

Grant Agreement

Used when **funding is being provided** to an organization to carry out specific work. The funder sets the terms, and the recipient agrees to use the money as specified.

Letter of Intent

A preliminary, non-binding document that signals **interest in entering a partnership or agreement**. It says "we intend to work together" but doesn't commit either party to specific terms yet. It's a first step, not a final agreement.

How to tell them apart on the exam:

Long-term partner, mutual benefit, discounted services → **MOU**. Specific deliverables with legal accountability → **Formal contract**. Money being given to carry out work → **Grant agreement**. Preliminary expression of interest → **Letter of intent**.

Knowledge Checks:

1. A local hospital has been a long-standing partner with a community health organization. The hospital agrees to provide meeting space and staff time at a reduced cost in exchange for co-branding on program materials. What type of agreement is most appropriate?
2. A health department hires an external evaluator to conduct a program evaluation with specific deliverables due on set dates. What type of agreement is most appropriate?
3. Two organizations meet to discuss a potential collaboration and write a document stating their shared interest in working together, without committing to specific terms. What is this document?

HIRING CONSULTANTS AND CONTRACTORS

When a health education specialist needs to bring in an outside consultant — usually for evaluation — the exam tests whether you know the correct first step.

The first step:

Before anything else, you need a **written agreement outlining the work needed**. This document defines the scope: what the consultant will do, what the deliverables are, what the timeline is, and what the expectations are. You don't start with an ethics agreement (that may come later), you don't start with extra budget for unexpected costs (that's a budget decision, not a first step), and you don't start with an implementation plan from the consultant (they can't plan until they know the scope).

Exam tip:

The distractors usually include reasonable-sounding things that happen later in the process — ethics agreements, implementation plans, budget contingencies. But the **first** step is always defining the work in

writing. You can't do anything else until both parties know what the job is.

Knowledge Checks:

1. A program director wants to hire an external consultant to evaluate a youth mentoring program. What should happen first?
2. A consultant is brought in to evaluate a community health program. The consultant immediately asks for an implementation plan. Is this the correct first step? Why or why not?

USING A VARIETY OF TRAINING STRATEGIES

When health education specialists conduct training programs, using a variety of resources and strategies is considered a best practice. The exam tests whether you know the primary reason for this.

The reason:

The primary reason to use a variety of strategies is to **ensure that all the necessary material is covered**. Different content requires different delivery methods. Some concepts are best taught through lecture, others through demonstration, others through hands-on practice. Using multiple strategies means you're matching the method to the content so nothing gets missed.

Common distractors:

The exam may offer "to accommodate funders' expectations" (that's not a training design principle), "to reduce costs" (variety doesn't necessarily reduce costs), or "to reach as many learners as possible" (that sounds good but the primary reason is about content coverage, not audience reach). The answer is about ensuring **all material gets covered effectively**.

Knowledge Checks:

1. A health education specialist is planning a two-day training on motivational interviewing techniques. Why should the specialist use a mix of lectures, role plays, and case studies rather than just lectures?
2. What is the primary reason for using a variety of resources and strategies when conducting training programs?